



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ام هاشم حسين عبدالعزيز محمد

العنوان : 0

الحى / المدينة : البدرشين

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 09-10-2026

رقم البطاقة : 27601242100986 رقم التليفون : 01062425819

العمر (تاريخ الميلاد) : 24-01-1976 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان

الوظيفة: -----
الوظيفة: -----
الوظيفة: -----

التوقيع: -----
التوقيع: -----
التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----

قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ام هاشم حسين عبدالعزيز محمد National ID: 27601242100986	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

03/10/2026

MDT FORM

Name:	Om Hashem Husein Abd El Aziz Mohamed	MRN:	3577
Age:	50 Y	ID:	27601242100956
Menopausal Sta:	perimenopausal	Phone:	01062423819
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	postoperative

Clinical Data:

R

L

BX: IDC, G II
ER: 8/8 PR: 8/8 HER2: 0 KI67:

Sonomammography:

Mets Workup:

History:

T2N1 received neoadj. CTX, then underwent LT MAM, pT2N1(2/10)

Recommendations:

for postoperative plan of management.

MDT Outcome & Plan

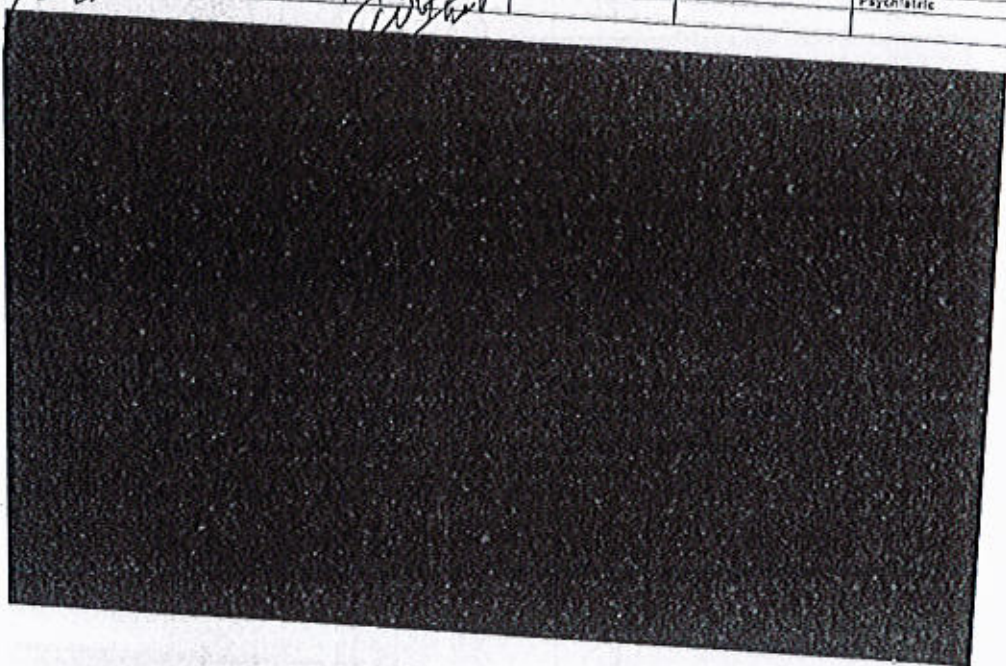
for PORT!!! Adj Hormonal tx! Femara +/- zoledex after new E2, PSH



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



جمهورية مصر العربية
بطاقة تحقيق الشخصية



ام هاشم
حسين عبدالعزيز محمد
الشويك القري
مركز البدرشين - الجيزة

١٩٧٩/٠٩/٢٤ ٢٧٦٠١٢٤٢١٠٠٩٨٦
ID7863203

٢٠١٩/٠٩ ٢٧٦٠١٢٤٢١٠٠٩٨٦



ربة منزل
أنثى
مستلمة
أرملة



البطاقة سارية حتى ٢٠٢٩/٠٩/١٦



Name:	ام هاشم حسين عبد العزيز محمد	
MR. :	3577	
Detailed Medical Report		
		السيد
تخية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The multidisciplinary team discussed the patient's portal (PORT) management and considered the use of aromatase inhibitor therapy with or without zoladex based on the estradiol (E2) level, which was recorded as "R". The patient was diagnosed with left-sided breast cancer, clinical stage cT3N1 invasive ductal carcinoma, grade 2, hormone-receptor positive (HR 8/8) and HER2-negative. Neoadjuvant chemotherapy (NACT) was administered, followed by left modified radical mastectomy (MRM) with final pathology pT2N1M0. The plan included postoperative radiotherapy (PORT) and initiation of hormonal therapy (HT) with Femara (letrozole) for three months, pending follicle-stimulating hormone (FSH) and estradiol (E2) results due to a "KARAR" issue. Estradiol level was measured at 10 pg/mL, consistent with post-menopausal status; therefore, zoladex was deemed unnecessary. The patient continued receiving PORT and Femara, with supportive treatment prescribed for three months (from 9 June 2026). The pharmacy was instructed to schedule the next clinic visit after the three-month period. Radiotherapy was completed in June 2026. The patient remains on Femara with supportive care for a total of three months, and the next follow-up appointment is scheduled for 4 October 2026. The patient reports new symptoms of blurred vision, headache, and instability while on Femara and supportive therapy. An MRI of the brain has been ordered to evaluate these neurologic complaints.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
Reham Ahmed Fathy Ibrahim		الطبيب المعالج :
يعتمد ,		

MDT FORM 18 / 1 / 2026 G

Name	ام هاشم	MRN	104/2023
Age	49	ID	257.15891.97
Menopausal status	premenopausal	Phone	01062425819
NRS SCORE		Marital status	Wid
BMI		Family history	
CO morbidity	ve	REASON OF MDT	

Clinical Data

BX: Mixed lobular & Ductal R

ER: 8/8 PR: 8/8 HER 2: -ve Ki67:

Sonomammography:

Locally advanced left breast

Mets Workup:

ipsilateral breast
Axillary LNs

Received Neo-adjuvant chemotherapy

Recommendations:

4 cycles AC + 12 Wks Taxol

BSM →

For CESM

For possible L+MRM (clinically poor)

MDT OUTCOME & PLAN

for L+MRM

Small Breast
with lobular mass L
extend to 3:00
with overlying



Pre-chemotherapy
TNM
3 1 0

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذة / ام هاشم حسين عبد العزيز محمد	Request No.	: 13009613
Patient No.	: 27601242100986	Request Date	: 04/02/2025
Path No.	: SR-205/25	Verification Date	: 10/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

inical Data:
Left breast mass.

Nature of Specimen:
Block revision.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- GATA3: Positive.
- E-Cadherin: Negative in lobular component.
- ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 20% nuclear reaction of tumor cells.

Conclusion:
Left breast mass, block revision:
Mixed invasive duct and lobular carcinoma, Grade II.
IPT:

- GATA3: Positive.
- E-Cadherin: Negative in lobular component.

Verified by :
dr.esraa magdy

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Page 1 of 2

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Patient Name	: الأستاذة / ام هاشم حسين عبد العزيز محمد	Request No.	: 13009613
Patient No.	: 27601242100986	Request Date	: 04/02/2025
Path No.	: SR-205/25	Verification Date	: 25/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

-One paraffin block labeled by (21348) and two slides were received.

Microscopic Examination:

- * Histological type: Invasive mammary carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- * DCIS: Minimal insitu component.
- * Angioinvasion: No.
- * Perineural invasion: No.
- * Microcalcifications: Positive.
- * IPT:
- GATA3: Positive.

Diagnosis:

Left breast cancer, Slide revision:
Invasive mammary carcinoma. with minimal insitu component.
For IPT (E-Cadherin, ER, PR, Her2 & Ki67).

Signature
Prof. Dr. Lubna Omer.
Dr. Samia Ali.

Verified by :
Dr.Toka Medhat

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Om Hashem Hussein Abdel Aziz Mohamed	Date : 06/01/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 27601242100986	Pat. ID: 96676

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

breast palpable mass

Report:

- Left Breast:
 - Edema pattern.
 - Irregular dense mass is seen in the UOQ seen reaching the puckled thickened skin.
 - Ultrasound shows this mass as irregular heterogenous lobulated mass. It is seen occupying 2 o'clock measuring about 15 X 45 mm.
 - Multiple pathologically enlarged axillary lymph node are noted showing thick irregular cortices and effaced hila (level 1 and 2)
 - No supra or infraclavicular lymph node.
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Locally advanced left breast malignant mass and ipsilateral axillary lymphadenopathy for histopathological assessment (BIRADS 5)
- Normal sonomammography of right breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

Patient ID: 92410	Patient Name English: Om Hashem Hussein Abdalaziz Mohamed
Age: 49 years old	2026/01/06 10:16:04 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: known case of left mixed invasive lobular and ductal carcinoma GII

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: minimal background enhancement

ACR: b (scattered fibro-glandular densities)

LEFT BREAST:

- **Low energy (Non contrast) images:** retroareolar asymmetry with distortion, skin & nipple retraction.
- No suspicious microcalcifications.
- **Recombined (Post contrast) images:** faint retroareolar heterogeneous focal non mass enhancement , best appreciated in MLO view , it measures roughly 16x12x8mm.
- **Complementary US:** retroareolar irregular spiculated mass infiltrating the nipple with nipple retraction.
- Level I 1 **indeterminate** lymph node which shows focal cortical thickness up to 3mm with mild indenting hilum, however it reserve its kidney shape .

RIGHT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

LEFT BREAST: (BIRADS6 = pathologically proved malignancy) retroareolar pathological enhancing mass infiltrating the nipple the.

RIGHT BREAST: (BIRADS 1= normal)

Best regards,

Dr. Marwa Ramzy, MD, FRCR,

Name: Mrs. Om Hashem Hussein Abdelaziz

Date: 29-12-2025

Bilateral mammography with high resolution breast sonography revealed:

History: Pathologically proven left breast malignant lesion received NACT. No previous studies available for comparsion.

Findings:

Left breast.

- Heterogeneously dense which might obscure small lesions. (ACR c)
- Left retroareolar irregular spiculated dense mass is seen with associated skin and nipple retraction.
- **Ultrasound** demonstrated a hypoechoic irregular solid retroareolar lesion infiltrating the nipple, measuring 18x8 mm.
- Single indeterminate left level I axillary lymph node is seen showing focal cortical thickening measuring 4.4 mm.
- No evidence of suspicious infra or supraclavicular lymph nodes in the current study.

Right breast.

- Heterogeneously dense which might obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic
- Normal skin/nipple/areola complex.
- **No** suspicious axillary, infra or supraclavicular lymph nodes in the current study

Conclusion: -

Left breast pathological proven malignant lesion with nipple infiltration and left axillary indeterminate lymph node (BIRADS 6) for correlation with unavailable previous studies.

Normal right breast (BIRADS 1)

Best Regards,
DR. Yara Mohamed, MD

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	



Patient Name: Om Hashem Hussein Abdel Aziz Mohamed	Date : 04/02/2025
Exam : C.T CHEST	Age : 49 Year(s)
Medical ID: 104/2025	Pat. ID: 96676

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left breast cancer.

I Chest:

- Left breast UOQ area of asymmetrical density seen reaching the nipple associated with prominent ipsilateral axillary lymph nodes.
- Middle lung lobe calcified nodule mostly representing post granulomatous sequel.
- No sizable pulmonary masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is mildly enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Bulky uterus.
- Bilateral femoral sclerotic foci mostly bone islands.
- D10 sclerotic lesion measuring 7 mm.

Opinion:

- Reported case of left breast cancer revealed:
- No sizable pulmonary or hepatic deposits on non-contrast basis.
- D10 sclerotic lesion for bone scan.

Dr. Mohamed Mohsen, M.D.